YK7 Strategic Intelligence Platform

How to Deal with Narcissistic Patterns

From Understanding to Protection Without Harm or Exhaustion

English Review Copy



Dr. Yasser Kotb

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Book Information

How to Deal with Narcissistic Patterns

From Understanding to Protection Without Harm or Exhaustion

By Dr. Yasser Sayed Abdel Salam Kotb

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English translation for invited review and consideration of a foreword.

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Preliminary Note

This book provides practical educational material for managing responses to harmful practices and organizing personal and professional boundaries. It does not diagnose individuals or offer a treatment program, and it does not replace professional assessment or advice appropriate to a person's circumstances.

The examples are hypothetical and intended for illustration. The term 'narcissistic pattern' and the tools of the 'Dignity and Boundaries Balance' are used in the operational senses explained in the text, and are distinguished from established clinical terminology.

Chapter One
What Am I Dealing With

Describe the Behavior Before Naming It

You may hear a hurtful remark, face an unreasonable demand, or find yourself having to justify an ordinary need. Understanding begins by describing what happened before interpreting the personality of the person responsible. Instead of 'He is a narcissist,' write: 'He mocked my request in front of other people,' 'He refused to discuss a specific mistake and returned to my shortcomings,' or 'He made access to an essential resource conditional on my agreeing to something I do not accept.'

This description does not minimize the impact. It makes the incident easier to discuss and follow up. A broad label, by contrast, may combine different events into a single judgment and then lead you to approach every situation through that judgment.

A single act can cause harm serious enough to require immediate protection. Paying attention to repetition does not mean waiting for violence or a serious threat to recur before recognizing its impact. Repetition can help

explain the course of events when appropriate; it is not a condition for recognizing a person's right to safety.

Traits Patterns and Disorder

Psychological literature uses several expressions that do not mean the same thing. Narcissistic traits describe characteristics that may be present to varying degrees. 'Pathological narcissism' concerns maladaptive difficulties in regulating self-esteem and relationships with others. 'Narcissistic personality disorder' is a clinical diagnosis with requirements concerning an enduring pattern, its effects on functioning and relationships, and assessment across multiple contexts. The DSM-5-TR describes narcissistic personality disorder through features involving grandiosity, a need for admiration, and impaired empathy. A single feature or a personal disagreement is insufficient to establish the diagnosis. [1, 2]

In the literature, 'pathological narcissism' does not necessarily denote a separate diagnosis or an inevitable stage preceding the disorder. Presentations may range between overt grandiosity and fragile self-esteem, and some dimensions may overlap without everyday observation permitting a particular diagnosis. The book

therefore does not use this expression to judge people through its examples or worksheets. [2]

These are different levels of description and use, not an inevitable ladder on which everyone with a trait progresses to a harmful pattern and then to a disorder.

Here, 'narcissistic pattern' is an operational term within the author's framework. It is used to consider a group of recurring practices that may involve marked self-centeredness, disproportionate entitlement, or insufficient consideration of a behavior's effects on others. It helps organize responses, rather than identify an illness in someone who is not being assessed. Not all harmful behavior is narcissistic, and narcissistic traits do not necessarily imply harm or establish a disorder.

Level of consideration	Appropriate question	Limit of inference
A specific incident	What happened and what was its impact?	Describes the incident without establishing a personality pattern.
Recurring behavior	How did it recur, in what context, and how was it addressed?	Helps explain the course of the interaction.

Level of consideration	Appropriate question	Limit of inference
An operational reading of a pattern	Do recurring practices cluster around self-centeredness, entitlement, and insufficient consideration of others?	A revisable practical assessment, not a clinical judgment.
Diagnostic assessment	Are diagnostic requirements met following a professional assessment?	A professional responsibility requiring tools beyond this book's scope.

What Understanding Narcissistic Features Can Add

Understanding self-centeredness, entitlement, and sensitivity about one's image may help interpret some difficulties in receiving criticism or discussing the effects of behavior. However, such an interpretation remains a possibility, not access to someone's intentions or inner world. The literature recognizes the diversity of narcissistic presentations rather than reducing them to one form. [1, 2] Practical sources also discuss communication and boundaries in relationships marked by self-preoccupation. [3]

The practical approach proposed here is to define the subject of a discussion, distinguish respect for the person from agreement with their request, and review what they do after what they say. If a discussion repeatedly turns toward establishing status or seeking admiration, it may help to return to the specific responsibility that needs to be addressed. This is a suggestion for responding to events; it does not establish what the other person feels or guarantee their response.

What Behavior Alone Does Not Reveal

Silence, defensiveness, jealousy, difficulty apologizing, and concern with public image may occur in many contexts. Fear, exhaustion, communication difficulties, or differences in power may also be involved. These behaviors alone do not establish narcissism, and these possible explanations must not be used to dismiss their impact when they are harmful.

Separating description from interpretation protects both people. You can say that monitoring is draining you without being certain of its psychological source, refuse humiliation without proving a disorder, and seek help because you fear escalation without knowing every motive behind it.

This distinction does not require a neutrality that treats causing harm and being harmed as equivalent. The aim is to judge behavior clearly while keeping interpretations of the person within the limits of what you know.

Severity Repetition and Unequal Capacity

When there is no immediate danger, comparing the severity and recurrence of incidents can help. A limited disagreement may be repaired, while practices that appear minor can accumulate and have a substantial impact. This distinction supports a response proportionate to the actual effects.

The meaning of an available choice changes with inequalities in power and resources. A direct objection whose consequences an independent person can bear may expose someone who depends on the other person for housing, money, or care to additional risk. The quality of a boundary is not measured only by the boldness of announcing it, but by the suitability of the action, the person's ability to implement it, and the support available.

These are perspectives for assessing a situation, not diagnostic indicators or a newly developed scientific scale. Chapter Two addresses following change, and Chapter Four explains how to formulate a request, an agreement, and a boundary.

The Dignity and Boundaries Balance

The operational framework in this book rests on four connected questions:

1. Behavior: What action am I describing, and what can I verify?

2. Impact: What effects did it have on my dignity, safety, work, or relationship?

3. Boundary: What will I define concerning my participation, resources, or the way I am treated, within what I have authority to decide?

4. Action: How will I implement this boundary, what support do I need, and what are the costs and risks of the step?

Readers consider these questions in light of context, resources, and differences in power. Answers are not combined into a diagnostic score. Readers do not have to identify a set number of signs before their experience deserves consideration. The 'Dignity and Boundaries Balance' and its worksheets are organizing tools developed by the author, not standardized psychological tests.

Practical Case A Passing Incident or a Recurring Course

Hypothetical case: A colleague challenges Layla's proposal in a meeting, using a phrase she experiences as belittling. After the meeting she tells him, 'The proposal can be criticized, but the words you used about my professional ability upset me.' He apologizes, explains his objection specifically, and does not repeat the remark in later meetings.

These events support an understanding of a disagreement that was addressed. There is no need to classify the colleague for Layla to retain her right to professional respect.

In another version of the case, the mockery recurs, every objection is met with an accusation that Layla cannot accept criticism, and credit for the work is then given to someone else. Documenting events, clarifying responsibilities, and seeking professional support become more important. Even so, the response concerns workplace practice and does not require turning the colleague into a diagnosis.

Applying the 'Dignity and Boundaries Balance' to the second version: The observed behavior is repeated mockery and diversion from the objection. The impact

is reduced professional participation and confusion over credit for achievements. The boundary is refusing to let discussion of a proposal become a hurtful appraisal of the person. An appropriate action might begin with requesting written clarification of feedback and responsibilities, then using relevant workplace channels if needed, with regard to safety and authority. None of these steps requires diagnosing the colleague.

What the case teaches: How someone responds to feedback is part of understanding the course of events, but is not diagnostic evidence by itself. A new incident does not erase earlier events, and one event does not determine the rest of the relationship forever.

Chapter Summary

Worksheet One in the appendix can help organize a description of the incident.

Begin by describing a verifiable incident and assessing its severity, recurrence, and effects in light of unequal capacity. Appropriate protection is possible without a diagnosis or certainty about intentions. The next chapter considers apologies and actual behavioral change.

Insights from the References
The Limits of Inferring a Diagnosis from Behavior

Weinberg and Ronningstam's review presents narcissistic personality disorder as a pattern pervasive across multiple contexts and discusses assessment of self and interpersonal functioning alongside observable features. Assessment extends beyond observing a single behavior. [2]

In practice, a harmful incident can be described and its effects considered without diagnosing the person responsible. Seeking protection concerns what is happening; it does not depend on settling on a clinical label.

The 'Dignity and Boundaries Balance' is the author's operational framework for organizing questions. Citing the literature does not confer the status of a standardized psychological test upon it.

Source: Reference [2], the section on categorical and dimensional diagnosis.

Practical Appendix
Worksheets for Organizing Situations and Decisions

These worksheets help apply reading to a specific situation. You do not need to use them all, or keep them if doing so exposes you to danger. Choose the sheet that serves your need and write enough to understand the incident, without turning your life into continual monitoring of the other person.

The worksheets were developed by the author within the 'Dignity and Boundaries Balance.' The 'Practical Implementation Balance' is an organizing worksheet that turns the framework's questions into a review of capacity, resources, and safety before acting; it produces neither a total score nor a diagnostic judgment. These are organizing tools, not standardized tests, diagnostic models, or professional risk assessments. Completing them does not establish a disorder or guarantee a decision's outcome. The tables contain space for answers; use a separate sheet if more space is needed.

Worksheet One
Describe the Incident Without Jumping to Intent

Purpose: Distinguish what happened, what you felt, what you interpreted, and what needs verification.

Question or item	My answer
Context: When the incident occurred and what it concerned.	
Observed behavior: The act or words, and the limits of my recollection.	
Impact: Effects on my feelings, work, resources, or safety.	
Interpretation: What meaning did I assume? Do I know the intention or consider it likely?	
Missing information: What can I not determine?	
Response: What I did and what happened afterward.	

Question or item	My answer
Current need: Clarification, agreement, support, or urgent protection.	

Brief hypothetical example: During a meeting I was interrupted three times and told, 'You do not understand this issue.' My presentation was disrupted and I felt humiliated. I assumed deliberate exclusion but do not know the intention. I need to present the events and agree on discussing the proposal without personal attacks. If it recurs, I will review the relevant workplace support channel. The example clarifies the need without requiring certainty about intent.

References and Supporting Reading

How the References Are Used

Numbers in the text correspond to the sources below. Diagnostic and clinical references define the limits of terminology. Practical reference [3] informs discussion of communication and boundaries. These citations do not establish the effectiveness of every statement or exercise in this book.

World Health Organization reference [4] originally addresses healthcare providers caring for women subjected to intimate partner or sexual violence. The book draws on its safety and support principles while identifying differences between populations and contexts. Reference [8] supports the discussion of seeking help when distress persists and affects everyday life.

References [10] and [11] support general stress coping and worry-management principles in Chapter Three. The case, Worksheet Seven, and the wording of the questions are the author's practical construction. Citing those sources does not confer standardized-test status or proven effectiveness upon every step.

The situations, dialogues, and worksheets are educational formulations by the author. They are not presented as cases documented in these sources, standardized diagnostic or risk assessment tools, or substitutes for treatment or professional advice.

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merckmanuals.com

8. World Health Organization, Regional Office for the Eastern Mediterranean. (n.d.). Mental health and psychosocial support platform: Dealing with stress.

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9. American Psychiatric Association. (2024, January 30). What Is Narcissistic Personality Disorder?

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10. World Health Organization. (2020). Doing what matters in times of stress: An illustrated guide.

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who.int

11. National Health Service. (n.d.). Tackling your worries. Every Mind Matters.

General guidance for organizing worries and identifying what can be acted on, with attention to individual and contextual differences.

nhs.uk

The electronic source details and available links in this version were last checked on 31 October 2026. References [5-7, 9] support clinical definitions; they do not support diagnosing anyone through the book's examples.